

PROJECT 100

The Simplified Structure V2.0

Easy to read. Ready to present. Ready to publish.

The Complete Full Encyclopedia distilled into one clear system - now with clinical analysis in every section: mechanism, why each record is locked, what to monitor, and when to stop.

Read the system in 20 minutes, then run it one day at a time

V2.0 keeps every locked dose and status, then adds why the lock exists, what to watch, and when to stop and review.

Source: generated from **Project 100 Encyclopedia V2.0** (2026-10-03); the Encyclopedia governs.

For presentation

Pages 3 to 6 tell the story: target, return, system, measurement. Pages 7 to 10 run the day.

For daily use

Keep Chapter 4, the locked records, the margin dashboard and the checklists open.

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Mechanism: Dose, timing, tissue response and total load decide whether an input helps or costs.
Why locked: Confirmed, optional, paused, removed and inactive items stay separate so old pages cannot become new instructions.
Monitor: Log actual intake, sleep, digestion, pain and performance.
Stop rule: Symptoms needing medical care override the schedule and the target.

Reader contract: This is Bogdan's documented personal routine, not a prescription. Publication checks records and arithmetic; it does not grant clinical clearance.

100 lb of muscle is the north star; health is the boundary

Project 100 aims for **100 lb of reported skeletal muscle mass** with **body fat below 15%**. Those numbers are Bogdan's personal targets. They are not medical thresholds, guaranteed outcomes, deadlines, or reasons to train through illness.

90.76_{kg}

Current body weight
Age 46 - source profile 2026

88.2_{lb}

Skeletal muscle mass
February baseline

22.9%

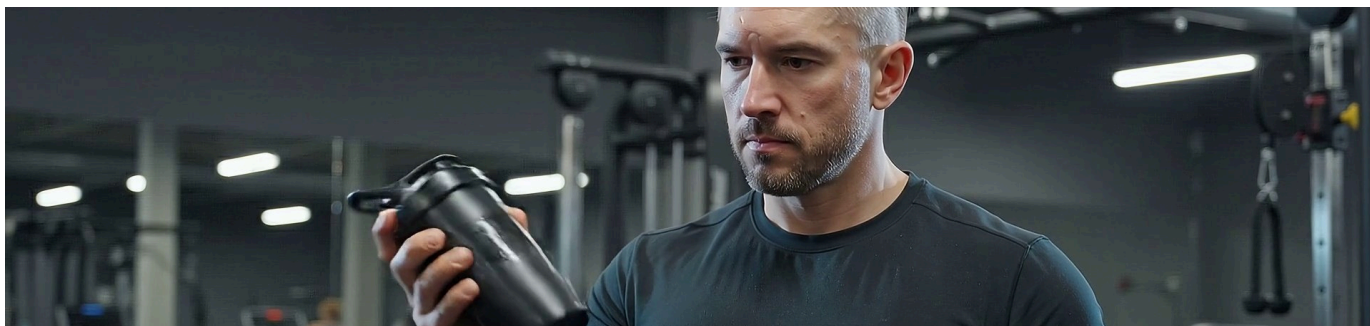
Body fat, February baseline
Target: below 15% over time

What 100 means, precisely

- **Skeletal muscle mass**, not lean body mass and not total weight. The three cannot be swapped.
- Bioimpedance estimates muscle from body water and an algorithm. It does not weigh tissue directly.
- Same device and similar conditions, then judge the trend over weeks.

The old motto, kept in its place

"Train Like Two. Eat for Two." remains historic Project 100 branding. It means sustained effort, not double workouts or double calories for everyone. Food, load and rest are matched to the work actually done.



The target is personal. The trend, function and health record are the judges.

Mechanism: Bioimpedance is sensitive to intracellular and extracellular water, glycogen, electrolytes, recent exercise and meal state.

Why locked: Standardized waking, post-void, pre-intake measurement is the only fair comparison condition in this system.

Monitor: Track weight, waist, strength, sleep, digestion and pain beside the estimate; review body composition quarterly.

Stop rule: Do not change food, pills or training load because of one dramatic reading.

Five hard years, one clear return

Underneath the tables, Project 100 is a return story: heavy drinking, stopping, then rebuilding a daily structure that could hold training, digestion and sleep again.

Period	Record
2020-2025	Heavy alcohol use. This edition does not invent a single exact chronology for older interruptions.
2025	Cessation. Attention turns back to health. This is the Pivot: the point where structure starts replacing drift.
Feb 2026	Baseline recorded: 197.1 lb total weight, 88.2 lb skeletal muscle mass, 22.9% body fat.
Mar 2026	Laboratory baseline on 17 March 2026. Historical values only; they predate the later stack and cannot certify it.
May-Sep 2026	Routine rebuilt and corrected: Zinc Supreme removed, separate copper paused, boron fixed at one capsule.



The Pivot is not a slogan. It is the decision to rebuild a repeatable day.

Mechanism: Detraining reduces strength, work capacity and routine tolerance; retraining restores them gradually when load, food and sleep are repeatable.

Why locked: Excavation comes before intensification because a tolerable routine is the asset that survives bad weeks.

Monitor: Watch sleep duration, morning readiness, joint symptoms, digestion and whether sessions can be repeated without a flare.

Stop rule: New weakness, persistent numbness, chest pain, fainting or bowel/bladder change with back pain means stop and seek assessment.

Intake x Vector x Time: three words that organize everything

INTAKE x VECTOR x TIME is an organizing metaphor, not a physiological equation. A more elaborate schedule does not automatically improve absorption or adaptation. Used well, it makes a day traceable.

Term	Practical meaning	Example
Intake	Food, fluids and supplements actually consumed	Breakfast, drinks, meal pills
Intake vector	A numbered beverage recipe with ingredients and timing	V4 RELOAD after morning gym
Movement class	A training action and its loading demands	Supported row, machine press
Chronometer	Sequence and timing window	Whole R-ALA capsule before V4
Hardware lock	A symptom or clinical constraint that comes first	Worsening hand weakness needs assessment

Training effort levels used in this book

PRIMARY LEVEL 1 (WEIGHTED, GREEN)

SECONDARY LEVEL 2 (DYNAMIC, BLUE)

TERTIARY LEVEL 3 (ROTATION/ACCESSORY, YELLOW)

Level 1 carries the main progressive work. Level 2 adds dynamic support work. Level 3 rotates accessories and keeps the week complete without turning every exercise into a maximum effort.

Mechanism: Mechanical tension, adequate protein, sleep and repeatable practice drive adaptation; labels only organize those inputs.

Why locked: The system is locked to actual consumption. An unused recipe contributes zero dose.

Monitor: Record branch, recipes used, main lifts, symptoms and sleep. Review the week, not a single heroic session.

Stop rule: If the routine cannot be repeated safely, simplify before adding another variable.

Trends you can trust, readings you should not chase

Compare like with like

- Same device, similar time of day, hydration, meal state and recent exercise.
- A few comparable weigh-ins describe a week better than one dramatic morning.
- Track weight, waist, performance, sleep, digestion and pain alongside the estimate.
- Quarterly body-composition review is a workable project cadence, not a medical order.

Laboratory integrity

- March 2026 labs predate the May supplement stack. They cannot certify it.
- Use the **biotin hold** rule: tell the laboratory the exact biotin intake and follow its assay-specific hold instruction.
- For kidney interpretation on creatine, ask about cystatin C rather than relying only on creatinine-based estimates.
- Use the **K+/Na+/CrCl** panel when electrolyte or kidney context needs review.

Date	Branch	Actually consumed	Sleep	Symptoms / performance
Each day	Training / Rest	Recipes, unusual servings, caffeine	Duration, awakenings	Digestion, pain, strength notes

Reported fluids average 4 to 5 litres daily including vector liquids. Night-time urination is about two to three times on gym days and about once on rest days. A timing and volume diary is more useful than adding another kidney product.

Mechanism: Creatine can raise serum creatinine without the same change in true filtration; hard training can also raise CK, LDH and AST from muscle turnover.

Why locked: The panel is locked to comparable conditions and disclosed supplement use, because a clean number from a confounded test is still a bad decision input.

Monitor: Before labs: disclose biotin and creatine, record last hard session, and use morning fasting timing when hormones are being assessed.

Stop rule: Do not diagnose kidney, liver or hormone status from one confounded value. Repeat or re-test with the clinician's chosen markers.

4.1 Morning

The morning decides which branch the day is on. Sequence matters more than forcing the exact minute.

WAKE Morning pack as recorded	V3 + BREAKFAST Kasha, tea, real meal first	DECIDE Training or rest branch	LOG Sleep and symptoms
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Morning lock	Current record
Boron	boron 1 x 3 mg - one 3 mg capsule. Do not multiply it across products without a full total check.
Dedicated Zinc Supreme removal	Dedicated Zinc Supreme removal: Zinc Supreme is removed and contributes zero. The remaining zinc record is zinc 25 mg/day from the multivitamin on the training grid.
Copper status	Separate copper is copper paused . Paused means zero while paused; it is not a hidden daily dose.
Vitamin D lock	D3 125 mcg/5,000 IU 2x/week . This is a twice-weekly lock, not a daily escalation prompt.

Mechanism: Zinc, copper and boron interact through absorption and long-term mineral balance; more separate products make the total harder to see, not automatically better.

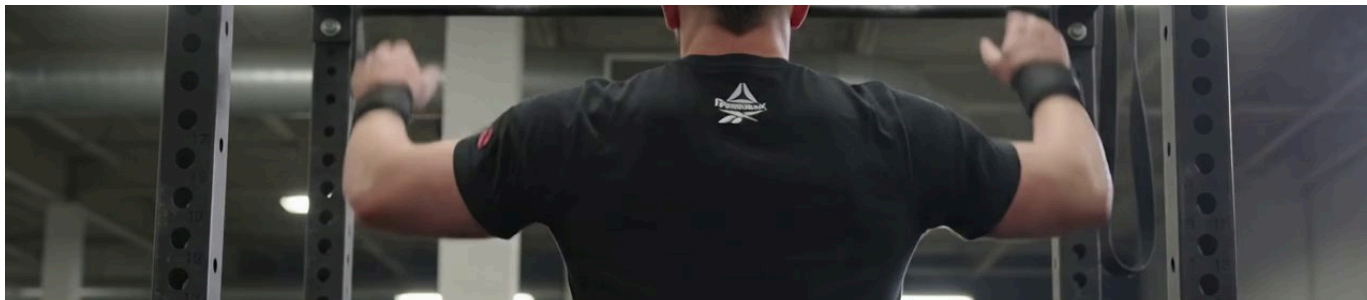
Why locked: Zinc Supreme removal, copper paused and boron 1 x 3 mg keep the mineral record auditable.

Monitor: Check the physical labels at each quarterly audit. Record actual branch and actual capsules.

Stop rule: New numbness, gait change, unusual fatigue or persistent GI upset triggers clinical review rather than mineral self-correction.

4.2 Training Day Window A+B

Window A is the pre-gym and gym block. Window B is the reload and meal block that closes the training stimulus.



Morning training branch: supported, repeatable work, logged the same way each time.

Window	Locked record	Why it is locked
A - Ignition	V1 before gym; citicoline 250 mg in the morning record; IGNITER exposure stays counted.	Caffeine near 373 mg from IGNITER leaves little room before the 400 mg daily reference once tea is added.
A - Session	V2 during the session when consumed; dates and fluids are actual intake, not assumed intake.	Intra-workout fuel supports the work done; it is not a reason to extend a bad session.
B - Reload	Whole R-ALA capsule before V4. V4 uses one 36 g BioSteel scoop: 12 g protein, 19 g carbohydrate, 125 kcal from BioSteel alone.	R-ALA is swallowed whole to limit mucosal irritation and breakdown; the one-scoop portion prevents silent doubling.

Mechanism: Caffeine blocks adenosine receptors and has a multi-hour half-life; R-ALA can affect glucose handling and must be treated as an active input, not a harmless add-on.

Why locked: The A+B order prevents the commonest errors: doubled scoops, uncounted caffeine and capsules emptied into shakes against the record.

Monitor: Track caffeine total, sleep latency, tremor, sweating, dizziness, stool response and session performance.

Stop rule: Stop and seek medical advice for hypoglycemia-type symptoms such as tremor, sweating, confusion or faintness; do not train through them.

4.3 Evening 20:00

The evening close is a landing, not a second launch. The 20:00 reference point keeps the night routine visible without turning it into a deadline.

Evening item	Locked handling
V10 night close	800 mL hibiscus tisane with glutamine, glycine and creatine HCl as recorded, when actually consumed.
Magnesium glycinate	Training-day evening record only in the current grid; rest-day handling stays smaller by design.
Optional powders	Optional half Medi-C and rare Hey Relax remain optional or rare. They are never daily requirements and never silent defaults.
Evening PUMP branch	Inactive. Do not combine an evening PUMP idea with the morning IGNITER record.

Close the loop

Log dinner, fluids after 20:00, expected night urination, and whether the day was truly training or rest.

Protect sleep latency

Late caffeine, large fluid boluses and unresolved GI load show up first as broken sleep.

Mechanism: Evening magnesium and vitamin C can add osmotic load in a gut without a gallbladder; fluid timing also changes night urination and sleep continuity.

Why locked: The 20:00 close is locked so optional items cannot drift into nightly escalation.

Monitor: Watch sleep onset, awakenings, stool urgency, reflux or nausea, and next-morning readiness.

Stop rule: Recurrent watery stools, weight loss, severe pain or persistent nausea needs clinical review, not a stronger night powder.

4.4 Rest

A rest day is smaller on purpose. It is not a failed training day and it does not import the training pill grid to feel complete.

Keep on rest days

- Morning pack as recorded and boron 1 x 3 mg
- Digestive enzymes with actual main meals
- Intermittent LactoBif as recorded
- V3, V5 and V10 only when actually consumed
- **S016 ENDURANCE PILL (STANDARD)** belongs here: rest days only

Skip on rest days

- Training pill grid, including the training multivitamin pattern
- V1, V2 and V4 training recipes
- Evening PUMP alternative (inactive branch)
- Any attempt to escalate D3, Alpha-GPC or Piracetam to fill the gap

Retired timing myths

- No universal mineral firewall: calcium later in the day does not cancel zinc in a morning multivitamin. Dose and duration matter more than blanket hour gaps.
- No mandatory hot-drink rule. Use a comfortable temperature; very hot drinks are a concern, not a recovery advantage.
- Hot liquid is not replacement bile. The liver keeps making bile after gallbladder removal.

Mechanism: Recovery is when adaptation consolidates; cholinergic and stimulant load on a rest day can cost sleep without adding training benefit.

Why locked: The rest branch is locked smaller so recovery does not compete with a full grid.

Monitor: Track whether rest actually restores: sleep, soreness trend, resting readiness, digestion and mood.

Stop rule: If rest days repeatedly feel wired, nauseated or broken by GI symptoms, review stimulants, minerals and meal fat before adding anything.

V1 to V10, each with one job

Code	Name	Job in the system	Branch
V1	IGNITION	Pre-workout recipe before morning gym, about 06:00	Training
V2	REFUEL	Intra-workout fuel during the session, about 09:30	Training
V3	RECOVERY	04:00 morning recipe used on both branches when consumed	Both
V4	RELOAD	11:00 post-workout shake with one 36 g BioSteel scoop base, after whole R-ALA	Training
V5	TRANSPORT	16:00-16:30 rest-day recipe when consumed	Rest
V10	SHUTDOWN	18:30 night close, 800 mL hibiscus tisane, when consumed	Both

Names, timings and formulations follow Encyclopedia Part III. V6 to V9 do not appear as active branches, and the evening PUMP alternative is preserved only for reference and strictly inactive. Count each actual dose once. An unused recipe contributes zero.

Count once

Actual consumption is the dose record.

Labels first

Brand and label version matter as much as product name.

Status matters

Active, optional, rare, paused, removed and inactive are different statuses.

Mechanism: A vector changes osmolarity, carbohydrate load, mineral load and timing all at once; the recipe name is not the active ingredient list.

Why locked: Each vector has one job so overlapping recipes cannot quietly duplicate the same exposure.

Monitor: Record which vectors were actually consumed and any GI, sleep or performance response that follows.

Stop rule: Loose stools, reflux, tremor or sleep disruption after a vector means review that recipe before the next identical day.

S016-S023: the exact records that cap the stack

Record	Locked V2 record
S016	S016 ENDURANCE PILL (STANDARD) citicoline 250 mg, DMAE 100 mg, alpha-GPC 300 mg, huperzine A 200 mcg on rest days only
S017	S017 ENDURANCE GIGA 61.2 g
S018	S018 TRINITY 1.5 tabs Accutane-free
S019	S019 BIG 100 61.5 g - with BIG 100 Berberine 250 mg stated explicitly
S020	S020 YOHIMBE PLUS 1 cap
S021	S021 METAMUCIL 1 cap
S022	S022 DAILY+ 2 caps
S023	S023 capped at 21 Supp + 2 Nuance = 23 total, priority Nuance

Grid rule

The **21+2 grid** is exact: 21 Supp plus 2 Nuance equals 23 total. The ceiling is **23 pills/tabs max not doses** - count pills/tabs, not dosing occasions.

Alpha-GPC cap above all

Alpha-GPC cap above all: the locked Alpha-GPC cap outranks every stacking preference. Do not add extra Alpha-GPC around S016.

Mechanism: Citicoline, Alpha-GPC, DMAE and huperzine all push cholinergic signalling; stacking them can move from focus support to headache, GI upset, vivid dreams or sleep disruption.

Why locked: S016 is rest-day only and the grid is capped so a good day cannot become an accidental escalation day.

Monitor: Monitor headache, nausea, loose stool, dream intensity, sleep quality, heart-rate feel and next-day focus.

Stop rule: Stop the cholinergic stack and review if headache, GI distress, agitation or sleep disruption repeats; seek care for severe symptoms.

Food first, totals only when they are actually known

What is documented

- **Breakfast:** about 80 g dry roasted kasha with almond butter and zero-sugar syrup, followed by Earl Grey tea.
- **Training fuel:** carbohydrates in the pre-gym meal and intra-workout dates support the session.
- **Post-workout:** V4 one-scoop BioSteel base, 19 g carbohydrate from BioSteel alone.

What is not yet claimed

- Old calorie and protein figures are historic planning data, not the verified present diet.
- A complete whole-day food audit remains separate and unfinished.
- No fixed rest-day carbohydrate percentage cut is imposed on the current routine.

Food Vector exactly add MSG/completion caveat: when a food vector is written into the day, add the MSG/completion caveat - sauces, seasonings and restaurant items can carry MSG and sodium that a home recipe line misses, and every food total stays partial until the whole day is actually logged. Do not present a partial vector total as a completed diet.

Protein

Adequate protein supports training, but more does not produce proportional gains indefinitely. Collagen is not equivalent to complete dietary protein for muscle building.

Fats without a gallbladder

Smaller fat portions spread through the day may be better tolerated than one large fatty meal. That is a tolerance strategy, not a universal fat ceiling.

Carbohydrate timing

Night carbohydrate is not automatically stored as fat because of the clock. Total intake, activity and individual response matter.

Mechanism: Without a gallbladder, bile flows continuously rather than being stored and released in a bolus; large fatty meals are therefore harder to match with bile and enzymes.

Why locked: Food records stay honest: documented meals are real, partial totals are labelled partial, and the MSG/completion caveat stays attached to food vectors.

Monitor: Track meal size, fat load, enzyme use, stool form, urgency, nausea and whether symptoms follow specific sauces or restaurant meals.

Stop rule: Severe abdominal pain, vomiting, jaundice, fever or persistent steatorrhea needs medical review.

Supported, tolerable, repeatable - then progressive

Evaluate every exercise by purpose, setup, tolerance and ability to progress. Support can reduce a troublesome demand; it does not remove all joint or spinal loading.

PRIMARY LEVEL 1 (WEIGHTED, GREEN)

SECONDARY LEVEL 2 (DYNAMIC, BLUE)

TERTIARY LEVEL 3 (ROTATION/ACCESSORY, YELLOW)

Exact labels: PRIMARY LEVEL 1 (WEIGHTED, GREEN) | SECONDARY LEVEL 2 (DYNAMIC, BLUE) | TERTIARY LEVEL 3 (ROTATION/ACCESSORY, YELLOW)

Level	Role	Project 100 examples
Level 1	Main weighted work that carries progression	Smith machine chest press, leg press, lat pulldown when tolerated
Level 2	Dynamic support work that builds volume without stealing recovery	Dumbbell pair rows, seated cable rows with controlled range
Level 3	Rotation and accessory work that completes the week	Band shoulder press, mobility and targeted accessories



Between sets is part of the exercise: set up, breathe, repeat the same way.

Mechanism: Mechanical tension drives muscle adaptation, while spinal shear, elbow angle and grip choice decide whether the dose is tolerable enough to repeat.

Why locked: Supported rows and presses are preferred when they preserve output with less symptom cost; support is a tool, not a guarantee.

Monitor: Log exercise, load, repetitions, intended effort, pain before/after and any numbness or grip change.

Stop rule: Stop the movement for progressive weakness, persistent numbness, sharp joint pain or symptoms that worsen across sets.

Constraints come before the schedule

Priority order: a symptom needing medical attention first, then known movement and digestive constraints, then recovery, dose accounting and the day's schedule. A pill grid cannot override that order.

Lock	What is reported	What this edition does not claim
Spine L4-S1	Back complaints in source guides. Floor deadlift prohibition is a personal constraint.	A belt, rack height or sleeping angle guarantees unloading or disc recovery.
Ulnar elbow / grip	Left-arm symptoms with perceived improvement. Imaging concerns elbow and shoulder tissues.	Straps or grips decompress a nerve in every lift, or imaging proves nerve recovery.
Digestion	Post-cholecystectomy. Enzymes reported helpful.	Gallbladder removal stops bile production, or enzymes prove pancreatic insufficiency.
Sleep / endocrine	Evening routine is training-day weighted; rest days stay smaller.	A night powder guarantees endocrine reset or testosterone correction.

Seek assessment promptly for: fainting, chest pain, severe confusion, new bladder or bowel dysfunction with back symptoms, progressive weakness, persistent numbness in ring and little fingers, jaundice, recurrent watery stools with weight loss, or significant pain.

Monitor: symptom location, duration, provoking movement and whether it is improving or spreading.

Stop rule: urgent signs override training, supplements and presentation deadlines.

Exercise selection follows the actual examination and symptom history. A general book cannot clear a specific spine, elbow or shoulder for a particular lift.



The lock is not fear. It is the rule that keeps next month possible.

Gallbladder fasting $\text{Ca}^{2+}/\text{K}^{+}$ 3-block matrix

Use this long-view matrix when a fasting window, a large meal and electrolyte totals meet in the same day.

Block	What happens	Locked action
Block 1 - Fasting	Without a gallbladder, bile is not stored for a timed release; it trickles continuously. A long fast can leave the first meal exposed to a mismatch between fat load and available bile/enzyme support.	Do not force a fat or fluid flush. Note nausea, right-sided pain, reflux and stool changes during the fast.
Block 2 - Break-fast meal	The first meal sets the GI tone for the day. Large fat, magnesium and vitamin C loads can combine into urgency or loose stool.	Break the fast with a tolerable fat portion, enzymes with the actual meal, and fluids spread rather than forced.
Block 3 - Electrolyte review	$\text{Ca}^{2+}/\text{K}^{+}$ totals come from food, drinks, vectors and pills together. Calcium and potassium cannot be judged from one product line.	Review the whole-day $\text{Ca}^{2+}/\text{K}^{+}$ picture and use the $\text{K}^{+}/\text{Na}^{+}/\text{CrCl}$ panel when kidney or electrolyte context is in question.

Mechanism: The osmotic chain is additive: magnesium, vitamin C, fat load, bile flow and fluid volume can each be tolerable alone and disruptive together.

Why locked: The 3-block matrix is locked because symptoms after a fast are often misread as a single-ingredient failure.

Monitor: Track fasting length, first-meal fat, enzyme use, stool form, urgency, nausea, cramps and fluid timing for three comparable days before changing the system.

Stop rule: Severe pain, vomiting, fever, jaundice, blood in stool or dehydration signs mean stop the experiment and seek care.

The stack risk is combinatorial, not single-ingredient

Cascade	Mechanism	Locked response
Cholinergic load	Citicoline, Alpha-GPC, DMAE and huperzine converge on acetylcholine signalling.	Respect S016 rest-day placement and the Alpha-GPC cap above all.
Piperine clearance	Piperine can inhibit CYP3A4 and P-gp pathways, changing how some drugs and compounds are cleared.	Disclose the full stack before prescriptions, surgery or dental procedures.
R-ALA glucose effect	Alpha-lipoic acid can lower glucose and, rarely, is linked with insulin autoimmune reactions in susceptible people.	Swallow whole before V4 as recorded; stop and seek advice for tremor, sweating, confusion or faintness.
Chromium caution	Chromium Wide-Range Caution: 200 mcg picolinate + Gluconated Stable Window merged/restore around 600 mcg/day total elemental	Treat chromium as a wide-range accounting problem, not a reason to chase glucose effects.

Chromium lock: Chromium Wide-Range Caution: 200 mcg picolinate + Gluconated Stable Window merged/restore around 600 mcg/day total elemental. **Freeze line:** freeze D3/Alpha-GPC/current Piracetam stress gate - do not escalate during stress, poor sleep or symptom noise.

Encyclopedia Part IV names five decisive cascades: piperine drug-clearance inhibition, the cholinergic pressor stack, the alpha-lipoic acid to insulin autoimmune pathway, the magnesium plus vitamin C osmotic stack without a gallbladder, and the stimulant stack at the caffeine ceiling. Risk concentrates in those combinations, not in single ingredients.

Stress gate freeze

Use the freeze plainly: **freeze D3/Alpha-GPC/current Piracetam stress gate** and do not escalate during stress, poor sleep or symptom noise.

Disclosure rule

Before labs, procedures or new prescriptions, disclose biotin, creatine, caffeine, botanicals, minerals and the full S016-S023 record set.

Mechanism: Interactions multiply when absorption, clearance, glucose, blood pressure and GI tolerance are pushed in the same direction on the same day.

Why locked: The freeze exists because stress and poor sleep distort the very signals used to judge whether a dose is helping.

Monitor: Monitor resting heart rate feel, blood pressure when available, sleep latency, tremor, headache, GI output and training readiness.

Stop rule: Stop and review for repeated headache, agitation, hypoglycemia-type symptoms, palpitations, rash, jaundice or persistent GI distress.

Known totals are flags for review, not clearance

Exposure	Locked record	How to read it
Magnesium scenarios	Mg scenarios 341/446/646 mg	Base training load, then higher optional-powder scenarios. Compare supplement magnesium with the 350 mg supplemental reference and watch stool response.
Vitamin C	Vit C 452.9 to 2,452.9 mg	The low end is the base training record; the high end is the Medi-C scenario before food. The 2,000 mg reference makes the high scenario a review flag.
Zinc	zinc 25 mg/day	Training-grid multivitamin record after Zinc Supreme removal. Rest-day pattern differs because the training grid is skipped.
Vitamin D	D3 125 mcg/5,000 IU 2x/week	Twice weekly. Do not convert it into a daily habit during a stressful week.
Caffeine	IGNITER near 373 mg before tea	Tea adds more. Count all sources against the day, sleep and heart-rate response.
Biotin	biotin hold plus disclosure	Known training-day biotin is disclosed to the lab; hold only as the lab directs for its assay.

Locked margin line: Mg scenarios 341/446/646 mg; Vit C 452.9 to 2,452.9 mg.

Upper limits are comparisons

Exceeding a reference is not automatically acute toxicity.
Staying below one is not clinical clearance.

Unknowns block clearance

Unquantified organ-powder nutrients and incomplete food totals prevent a clean whole-day clearance statement.

Mechanism: Magnesium and vitamin C share an osmotic GI pathway; zinc and copper share a mineral-balance pathway; caffeine and cholinergics share a sleep and readiness pathway.

Why locked: Margins are locked beside the numbers so a total cannot be quoted without its limit, branch and scenario.

Monitor: Use the K+/Na+/CrCl panel when kidney or electrolyte interpretation matters, and record which scenario produced any GI response.

Stop rule: Persistent diarrhea, vomiting, palpitations, confusion or fainting is a stop-and-look-for-care event, not a tolerance test.

Name the observation, choose the next review

Codes are labels for observations, not diagnoses. They keep a recurring issue visible without forcing an old automatic explanation.

Code	Observation	Appropriate first review
ERR_SLEEP_01	Short or disrupted sleep	Sleep opportunity, caffeine, fluid timing, symptoms
ERR_DIGEST_01	Bloating, nausea or loose stools	Meals, surgery history, enzymes, magnesium, vitamin C
ERR_HYPO_01	Tremor, sweating, dizziness, confusion	Possible hypoglycemia; review R-ALA and seek medical advice
ERR_AXIAL_01	Back pain or guarding	Provoking movement and clinical restrictions
ERR_ULNAR_01	Numbness, weakness, elbow symptoms	Avoid provocation; progressive weakness needs assessment
ERR_INTAKE_01	Dose, serving or brand mismatch	Reconcile physical label and current branch; fix record errors immediately
ERR_DRIFT_01	Routine becoming hard to sustain	Simplify the plan and preserve recovery

Warning/output monitor: engage warning/output monitor every training week - warnings are symptoms and red flags; outputs are sleep, stool, performance, pain and readiness scores.

Mechanism: recurring outputs reveal whether the system is adapting or accumulating cost.

Why locked: a code without an action becomes decoration; an action without a code becomes guesswork.

Stop rule: fix definite record errors first, trial one change at a time when safe, and stop multiple exposures when safety requires it.

Weeks build months, months protect years

Microcycle

A short repeated training period. Old example: 5 to 10 days.

Mesocycle

Several weeks with a common purpose. Old example: 4 to 6 weeks.

Macrocycle

The longer horizon inside a multi-year plan.

The four-phase loop

1 EXCAVATION

Rebuild a tolerable routine

2 ACCUMULATION

Progress suitable work

3 INTENSIFICATION

Higher effort, adequate
recovery

4 CORRECTION

Fix a documented problem

- Labels describe emphasis, not proven physiological states. No fixed durations and no mandatory deliberate overreach.
- **Deload:** reduce demand while keeping purposeful activity. A planned recovery week does not restart the journey.
- **Quarterly audit:** recheck branch rules, serving sizes, new labels, symptoms, performance and food records.
- Progress is judged on comparable measurements, function and health over quarters, not single readings.

Mechanism: Tendon, ligament and joint tolerance adapt more slowly than motivation; periodization exists to keep load inside tissue capacity while muscle and skill improve.

Why locked: Phases are locked to review points so intensity cannot drift upward without a correction loop.

Monitor: Every quarter: compare body-composition trend, main-lift log, ERR code frequency, sleep and the K+/Na+/CrCl panel when clinically indicated.

Stop rule: If performance falls while pain, GI distress or sleep disruption rises, enter correction rather than intensification.

One page to run a day, a week and a quarter

DAILY - TRAINING

- 4.1 Morning record
- 4.2 Training Day Window A+B
- Whole R-ALA capsule, then V4 one scoop
- 4.3 Evening 20:00 close
- Log sleep, digestion, pain, performance

DAILY - REST

- 4.4 Rest branch
- S016 on rest days only
- Enzymes with actual meals
- V3 / V5 / V10 only if consumed
- No training pill grid

WEEKLY / QUARTERLY

- Compare comparable weigh-ins
- Review loads by movement level
- engage warning/output monitor
- Check repeating ERR codes
- Quarterly labels, branches, symptoms, food record

A 5-minute presentation script

1. Project 100 is my return: 2020 to 2025 heavy drinking, stopping in 2025, rebuilding through 2026. **2.** The target is 100 lb of skeletal muscle mass under 15% body fat, judged on trends, not single readings. **3.** The system is Intake x Vector x Time, split into a morning training branch and a smaller rest branch. **4.** Training is supported, tolerable and repeatable; food and supplements are recorded at actual doses with limits stated. **5.** Constraints come first, rest is part of the plan, and the record stays honest.

Mechanism: A checklist works because it reduces decision load on tired days; the clinical lens stays attached so a shortcut cannot become a blind spot.

Why locked: The daily, weekly and quarterly loops are locked at different speeds: execution daily, pattern review weekly, system audit quarterly.

Monitor: If the same ERR code appears three times, change the review from willpower to mechanism: dose, timing, meal, movement or sleep.

Stop rule: Close every presentation with the reader contract: documented personal routine, traceable rather than transferable as a prescription.

Photo placement map and glossary

Exact eight-photo mapping for the publishing package

Photo ID	Exact placement map
S024	S024=Ice water cold therapy
S025	S025=Pivot photo elsewhere
S026	S026=Smith machine chest press
S027	S027=Lat pulldown
S028	S028=Dumbbell pair rows
S029	S029=Seated cable rows
S030	S030=Band shoulder press
S031	S031=Leg press

Production note: use these source IDs when placing the corresponding exercise photographs in the website, ebook and print package. The portrait photographs integrated in this simplified edition document Bogdan in the gym; they are not substitutes for exercise-demonstration images.

Glossary

Term	Plain meaning
SMM	Skeletal muscle mass as estimated by bioimpedance. An estimate, not a direct weighing.
Chronometer	The sequence and timing of the day. Sequence over exact minutes.
Vector (intake)	A numbered recipe: V1, V2, V3, V4, V5 and V10 in the current system.
Hardware lock	A symptom or clinical constraint that takes priority over the schedule.
Known totals	Partial supplement or food label calculations only. Not a whole-diet measurement.
Conditional	Retained in the grid because removal was not confirmed. Not the same as endorsed.

Version control: Project 100 Simplified Structure V2.0, dated 2026-10-03. Generated from Project 100 Encyclopedia V2.0 (2026-10-03), the single master volume merging the Full Detailed Version and the Complete Clinical Analysis, using the clinical analysis method: mechanism, why locked, monitoring and stop rules in every section.

Publication note: Keep dates and units on every value so an old observation is never presented as a newly measured result.